OMELON® ENTERIC MICROENCAPSULATED CAPSULE 20MG

Ingredient(s):
Each capsule contains:
Omeprazole …………………………………………………………….............……....…. 20mg

Pharmacodynamic:
Omeprazole reduces gastric acid secretion through a unique mechanism of action. It is a specific inhibitor of the gastric acid pump in the parietal cell. It is rapidly-acting and produces reversible control of gastric secretion with once daily dosing. Omeprazole is a weak base and is concentrated and converted to the active form in the acid environment of the intracellulas canaliculi within the parietal cell, where it inhibits the enzymes H+, K+-ATPase, the acid pump.
This effect on the final step of the gastric acid formation process is dose-dependent and provides for effective inhibition of both basal acid secretion and stimulated acid secretion irrespective of the secretagogue.
Omeprazole has no effect on acetylcholine or histamine receptors and no clinically significant pharmacodynamic effects have been observed other than those explained by the effect of omeprazole on acid secretion.
Helicobacter pylori is associated with acid peptic disease including duodenal ulcer and gastric ulcer in which about 95% and 80% of patients, respectively are infected with this agent. It is implicated as major contributing factor in the development of gastritis and ulcer recurrence in such patients.
Recent evidence also suggests a causative link between *Helicobacter pylori* and gastric carcinoma.
Clinical evidence has shown that the combination of omeprazole with amoxicillin eradicates in excess of 50% of *Helicobacter pylori* isolates irrespective of metronidazole sensitivity, and that clarithromycin may be a useful alternative. Other antibiotics which have been used in combination with omeprazole in clinical trails showing a useful effect on *Helicobacter pylori* include tetracycline and metronidazole. One study using omeprazole 40mg daily, amoxycycline 1500mg daily and metronidazole 1200mg daily for 14 days achieved on overall *Helicobacter pylori* eradication rate of 89% (96% in metronidazole-sensitive patients)
During treatment with antisecretory medicinal products, serum gastrin increases in response to the decreased acid secretion. Also CgA increases due to decreased gastric acidity. The increased CgA level may interfere with investigations for neuroendocrine tumours.
Available published evidence suggests that proton pump inhibitors should be discontinued between 5 days and 2 weeks prior to CgA measurements. This is allow CgA levels that might be spuriously elevated following PPI treatment to return to reference range.

Pharmacokinetic:
Oral dosing with 20mg omeprazole once daily provides rapid and effective inhibition of gastric acid secretion with maximum effect within 4 days of treatment. In duodenal ulcer patients, a mean decrease of approximately 80% in 24-hr intragastric acidity is then maintained, with the mean decrease in peak acid output after pentagastrin stimulation being about 70%, 24 hrs after dosing with omeprazole. The inhibition of acid secretion is directly related to the area under the plasma concentration-time curve (AUC) but not to the plasma concentration at any given time.

Indication(s):
Treatment of duodenal and benign gastric ulcers including those complicating NSAID therapy. Treatment of reflux oesophagitis. Zollinger-Ellision syndrome. Eradication of *Helicobacter pylori*. Long term treatment for recurrent duodenal ulcer.

Dosage and Administration:
Duodenal and benign gastric ulcers: 20mg once daily for 4 weeks in duodenal ulceration or 8 weeks in gastric ulceration; in severe or recurrent cases increase to 40mg daily.
Maintenance for recurrent duodenal ulcer: 20mg once daily.
Reflux oesophagitis: 20mg once daily for 4 – 8 weeks
Zollinger-Ellison syndrome: Initially 60mg once daily; usual range 20-120mg daily (above 80mg in 2 divided doses), for as long as is clinically indicated.
H. pylori eradication:
Triple therapy regimens: Omelon 20mg, amoxicillin 1g and clarithromycin 500mg, all twice a day for 1 week; or Omelon 20mg, clarithromycin 250mg and metronidazole 400mg, all twice a day for 1 week; or Omelon 40mg once daily with amoxicillin 500mg and metronidazole 400mg, both 3 times a day for 1 week.
Dual therapy regimens: Omelon 40mg daily with amoxicillin 1.5g daily in divided doses for 2 weeks.
Renal and hepatic impairment: No dosage adjustment necessary.

Route of administration:
Oral

Contraindication(s):
Known hypersensitivity to omeprazole.

Side Effect(s) / Adverse Reaction(s):

1. Omeprazole is well tolerated and adverse reactions have generally been mild and reversible.
2. Skin: Rarely, rash and/or pruritus. In isolated cases, photosensitivity, bullous eruption, erythema multiforme and alopecia.
3. Musculoskeletal: In isolated cases arthralgia, muscular weakness and myalgia.
4. Central and Peripheral Nervous System: Headache. Rarely, dizziness, paraesthesia, somnolence, insomnia and vertigo. In isolated cases, reversible mental confusion, agitation, depression and hallucinations, predominantly in severely ill patients.
5. Gastrointestinal: Diarrhoea, constipation, abdominal pain, nausea/vomiting and flatulence. In isolated cases dry mouth, stomatitis and gastrointestinal candidiasis.
6. Hepatic: Rarely, increased liver enzymes. In isolated cases, encephalopathy in patients with preexisting severe liver disease; hepatitis with or without jaundice, hepatic failure.
7. Endocrine: In isolated cases, gynoecomastia.
8. Haematological: In isolated cases, leucopenia, thrombocytopena, agranulocytosis and pancytopenia.
9. Others: Rarely, malaise. Hypersensitivity reactions, e.g. urticaria (rarely) and in isolated cases angioedema, fever, bronchospasm, interstitial nephritis and anaphylactic shock, in isolated cases increased sweating, peripheral oedema, blurred vision, taste disturbance.
10. Gastrointestinal disorders
    Microscopic colitis: Frequency ‘not known’
11. <u>Subacute Cutaneous Lupus Erythematosus (SCLE)</u>
    Skin and subcutaneous tissue disorders
    Frequency 'not known': Subacute cutaneous lupus erythematosus
    <u>Interstitial Nephritis</u>
    Renal and urinary disorders: Interstitial nephritis
    <u>Hypomagnesaemia</u>
    Metabolism and nutritional disorders
    Frequency “not known”: hypomagnesaemia.
    <u>Fracture</u>
    Musculoskeletal disorders
    Frequency “uncommon”: Fracture of the hip, wrist or spine.
    <u>Clostridium Difficile Diarrhea</u>
    Infections & infestations: Clostridium difficile associated diarrhea.
    <u>Fundic Gland Polyps (Benign)</u>
    Gastrointestinal disorders
    Frequency “common”: Fundic gland polyps (benign)
    <u>Vitamin B12 Deficiency</u>
    Metabolic/Nutritional: Vitamin B12 deficiency

Effect on ability to drive or operate machinery:

Omeprazole is not likely to affect the ability to drive or use machines. Adverse drug reactions such as dizziness and visual disturbances may occur. If affected, patients should not drive or operate machinery.

Precaution(s) / Warning(s):

1. Use in pregnancy & lactation: There is no evidence on the safety of omeprazole in human pregnancy. Animal studies have revealed reduced litter weights. Avoid in pregnancy unless there is no safer alternative. There is no information available on the passage of omeprazole into milk or its effects on the neonate. Breastfeeding should therefore be discontinued if the use of omeprazole is considered essential.
2. Use in children: There is no experience on the use of omeprazole in children.
3. Use in elderly: Dose adjustment is not required in the elderly.
4. Impaired renal or hepatic function: Dose adjustment is not required in patients with impaired renal or hepatic function. Patients with severe liver disease should not require >20mg of omeprazole daily.
5. When gastric ulcer is suspected, the possibility of malignancy should be excluded before treatment with omeprazole is instituted, as treatment may alleviate symptoms and delay diagnosis.
6. Increased Chromogranin A (CgA) level may interfere with investigations for neuroendocrine tumours. If the patient (s) are due to have a test on Chromogranin A level, Omeprazol treatment should be stopped for at least 5 days before CgA measurements to avoid this interference (see Pharmacodynamic). If CgA and gastrin levels have not returned to reference range after initial measurement, measurements should be repeated 14 days after cessation of proton pump inhibitor treatment.
7. <u>Regular Surveillance</u>
    Patients on proton pump inhibitor treatment (particularly those treated for long term) should be kept under regular surveillance.
    <u>Subacute Cutaneous Lupus Erythematosus (SCLE)</u>
    Proton pump inhibitors are associated with very infrequent cases of subacute cutaneous lupus erythematosus (SCLE). If lesions occur, especially in sun-exposed areas of the skin, and if accompanied by arthralgia, the patient should seek medical help promptly and the health care professional should consider stopping Omeprazole. SCLE after previous treatment with a proton pump inhibitor may increase the risk of SCLE with other proton pump inhibitors.
    <u>Hypomagnesaemia</u>
    Severe hypomagnesaemia has been reported in patients treated with PPI like Omeprazole for at least three months, and in most cases for a year. Serious manifestations of hypomagnesaemia such as fatigue, tetany, delirium, convulsions, dizziness and ventricular arrhythmia can occur but they may begin insidiously and be overlooked. In most affected patients, hypomagnesaemia improved after magnesium replacement and discontinuation of the PPI.

For patients expected to be on prolonged treatment or who take PPI with digoxin or drugs that may cause hypomagnesaemia (e.g., diuretics), health care professionals should consider measuring magnesium levels before starting PPI treatment and periodically during treatment.

Fracture

Proton pump inhibitors, especially if used in high doses and over long durations (>1 year), may modestly increase the risk of hip, wrist and spine fracture, predominantly in the elderly or in presence of other recognised risk factors. Observational studies suggest that proton pump inhibitors may increase the overall risk of fracture by 10–40%. Some of this increase may be due to other risk factors. Patients at risk of osteoporosis should receive care according to current clinical guidelines and they should have an adequate intake of vitamin D and calcium.

Clostridium Difficile Diarrhea

Published observational studies suggest that PPI therapy may be associated with an increased risk of Clostridium difficile associated diarrhea, especially in hospitalized patients. This diagnosis should be considered for diarrhea that does not improve. Patients should use the lowest dose and shortest duration of PPI therapy appropriate to the condition being treated.

Vitamin B12 Deficiency

Daily treatment with any acid-suppressing medications over a long period of time (e.g., longer than 3 years) may lead to malabsorption of cyanocobalamin (vitamin B12) caused by hypo- or achlorhydria. Rare reports of cyanocobalamin deficiency occurring with acid-suppressing therapy have been reported in the literature. This diagnosis should be considered if clinical symptoms consistent with cyanocobalamin deficiency are observed.

Drug Interaction(s):

1. Omeprazole can delay the elimination of diazepam, phenytoin and drugs that are metabolized by oxidation in the liver. Monitoring of patients receiving warfarin or phenytoin is recommended and a reduction of warfarin or phenytoin is recommended and a reduction of warfarin or phenyotin dose may be necessary when omeprazole is added to treatment. However, concomitant treatment with omeprazole 20mg did not change the blood concentration of phenytoin in patients on continuous treatment with phenytoin. Similarly, concomitant treatment with omeprazole 20mg did change coagulation time in patients on continuous treatment with warfarin.
2. There is no evidence of an interaction with theophylline, propranolol, metoprolol, lidocaine, quinidine, amoxycilin or antacids, but interaction with other drugs also metabolized via cytochrome P-450 system cannot be excluded.
3. The absorption of omeprazole is not affected by alcohol or food.
4. Simultaneous treatment with omeprazole and digoxin in healthy subjects lead to a 10% increase in the bioavailability of digoxin as a consequence of the increased intragastric pH.
5. The absorption of some drugs might be altered due to the decreased intragatric acidity. Thus, it can be predicted that the absorption of ketoconazole will decrease during omeprazole treatment, as it does during treatment with other acid secretion inhibitors or antacids.
6. Plasma concentrations of omeprazole and clarithromycin are increased during concomitant administration.

Symptoms and Treatment for Overdosage, and Antidote(s):

There is no information available on the effects of overdosage in man. However, omeprazole is very specific in action and no particular problems are anticipated. Symptomatic and supportive therapy should be given as appropriate. Single oral doses of up to 160mg and total oral doses of up to 300mg/day have been given without adverse effects.

Shelf-Life:

2 years from the date of manufacture.

Storage Condition(s):

Keep in a tight container. Store at temperature below 25ºC. Protect from light and moisture.

Product Description & Packing(s):

A size #2 pale brown cap and pink body hard gelatin capsule, containing milky white pellets, and both cap and body impressed with a mark " YSP / OMCp ".

Blister packing 10's x 10, 7's x 2, 7's x 4, 14's x 10 and 7's x 50.

Y. S. P.®

Manufacturer and Product Registration Holder:
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